

****DISCHARGE SUMMARY****

****PATIENT DEMOGRAPHICS****

Name: Patient 39

Age: 70 years old

Sex: Male

MRN: 000-TEST-39

Date of Admission: 06/12/2026

Date of Discharge: 06/18/2026

Admission Type: Elective

****PRIMARY DIAGNOSIS****

Left total hip arthroplasty (THA) — cemented prosthesis for primary osteoarthritis

SECONDARY DIAGNOSES

- Gout, tophaceous, left foot — mild
- Benign prostatic hyperplasia with nocturia x2–3/night
- Hypertension, well-controlled
- Hyperlipidemia

****HOSPITAL COURSE****

Patient 39 was admitted electively on 06/12 for cemented left THA performed on 06/13 without intraoperative complications. Surgical approach was anterolateral; component positioning assessed intraoperatively via fluoroscopy — satisfactory alignment noted. EBL approximately 450 mL. Patient tolerated procedure well under spinal anesthesia.

Postoperatively, patient remained in-hospital for 5 days. Pain managed with multimodal regimen. Wound healing progressed appropriately — minimal serous drainage by POD#3, no erythema or induration. Sequential compression devices applied bilaterally; DVT prophylaxis initiated with enoxaparin 40 mg SC q.d. ×14 days.

PT/OT initiated POD#1: ambulation with walker, transfer training, ADL modifications per hip precautions. Patient demonstrated good effort and understanding of weight-bearing restrictions. Discharged to home with outpatient therapy.

****MEDICATIONS AT DISCHARGE****

Medication	Dose	Frequency	Duration
------------	------	-----------	----------

---	---	---	---
-----	-----	-----	-----

Enoxaparin (Lovenox)	40 mg	SC daily	14 days post-op
----------------------	-------	----------	-----------------

Oxycodone-acetaminophen	5/325 mg	PO q.4–6h PRN pain	Until adequate PO analgesia, ~2–4 weeks
-------------------------	----------	--------------------	---

Celecoxib (Celebrex)	200 mg	PO q.d.	Continue indefinitely
----------------------	--------	---------	-----------------------

Allopurinol	200 mg	PO q.d.	Continue — gout prophylaxis
-------------	--------	---------	-----------------------------

Lisinopril	10 mg	PO q.d.	Continue
------------	-------	---------	----------

Tamsulosin (Flomax)	0.4 mg	PO q.h.s.	Continue
---------------------	--------	-----------	----------

Aspirin	81 mg	PO q.d.	Continue
---------	-------	---------	----------

****WEIGHT-BEARING STATUS & HIP PRECAUTIONS****

****Weight-Bearing:**** Toe-touch weight bearing (TTWB) left lower extremity ×4 weeks, then advance to partial weight bearing (PWB) as tolerated per PT assessment. Full weight bearing

anticipated by 8–10 weeks post-op.

****HIP PRECAUTIONS — CRITICAL:****

- NO hip flexion >90 degrees (avoid bending at waist, low toilets, low chairs)
- NO internal rotation of hip
- NO hip adduction past midline
- Use elevated toilet seat, reacher, sock aid at all times
- Sleep with abduction pillow between knees
- Avoid crossing legs

****PHYSICAL THERAPY & OCCUPATIONAL THERAPY ORDERS****

- PT 3x/week ×4 weeks, then reassess: ROM, strengthening, gait training, stairs
- OT 2x/week ×2 weeks: ADL training, home safety, adaptive equipment instruction
- Home exercise program: quadriceps sets, glute sets, supine hip abduction, heel slides — q.i.d.